

Tel: 01772 523748
Email: dawn.nightingale@lthtr.nhs.uk

DEPARTMENT OF RENAL MEDICINE
Royal Preston Hospital
Sharoe Green Lane
Fulwood
PRESTON
PR2 9HT

CONSULTANT: DR A P DHAYGUDE

Our Ref: APD/dn/66970
NHS No: 450 114 1867

Clinic Date: 21 November 2024
Date Typed: 02 December 2024

Dr C M Gee
Berry Lane Medical Centre
Berry Lane
Longridge
Preston
PR3 3JJ

Dear Dr Gee

Re: Simon Boardman, D.O.B. 06.02.1958
3 Barclay Court, Barclay Road, Longridge, Preston, PR3 3JF

Primary Renal Disease: Presented with severe AKI in the context of bilateral severe pyelonephritis due to e-coli infection (October 2024).
Remains dialysis dependent (November 2024)

Problems: Hypertension, diagnosed in 1991
Gout (2020)

Date Commenced RRT: 19.10.24

Date Commenced HD: 19.10.24

Kidney Choices Review: Would prefer peritoneal dialysis. To be considered in January 2025 once he has been on holiday to Turkey in December and Christmas is over

Previous Transplant:
Current Medication:

Transplant List:

Allopurinol 200mg once a day
Atorvastatin 20mg at night
Apixaban 2.5mg twice a day for six month duration (see below)
Paracetamol as required

Suggested to start Renavit 450mg once a day

HD Regime: **Frequency:** Three times a week, four hours
HDF dialyser: FX600, A10
Blood Flow: 250 mls/min

Access Device:
Target Weight:
Actual Weight:
Latest Adequacy: **Kt/V:**

Dialysis BP: 120/69
Post Dialysis BP:
UF volume: Currently 500ml or less

URR 81% 07.11.24
Latest Blood Results:

Hb: 96	Albumin:	Potassium:
Corr. Calcium: 2.3	Phosphate: 1.82	Ferritin:
Iron:	Alk. Phos:	Bicarbonate:
TIBC:	Iron Sats: 14%	CRP:
PTH: 23		

I met this gentleman for the first time.

He was admitted with severe urosepsis and required ICU support for multi-organ failure. Various imaging showed severe right sided pyelonephritis and pyelonephritis affecting left lower pole of the kidney. He was discharged home as he remained dialysis dependent. He has recovered from infection and is passing some urine. Initially he felt that he had reduced urine output but, after asking, he thinks he might be passing more urine now although it is just a couple of times in the morning. His UF volume was around 1.5 litres and currently has gone to 400 to 500 ml and there is a possibility that urine output may have improved. I have explained to him that there is a very small possibility that his kidneys may recover enough to not need dialysis and can take several months.

He was seen by Kidney Choices Team and PD would be his preferred modality. He is planning to go to Turkey for dental clearance on 8 December and will be back around the middle of December. With Christmas around, he would prefer to have a PD catheter inserted in January and I have copied this letter to Vascular Access Team to list him accordingly.

I would be grateful to you for adding Renavit 450mg to his prescription which is a vitamin supplement for haemodialysis patients.

I have asked him to keep an eye on the urine output and inform the nurses. He will be reviewed again in a few weeks' time.

Yours sincerely

Electronically checked & signed.

DR AP DHAYGUDE
CONSULTANT NEPHROLOGIST

cc: Private & Confidential
Mr Simon Boardman
3 Barclay Court
Longridge
Preston
PR3 3JF

cc: The Renal Dialysis ~~Sisters~~ via email
Kidney Choices ~~Team~~ via email
Vascular Access ~~Team~~ via email

Important information for our patients and their families/friends/carers:

- If you do not receive notice about your next appointment when expected *please* contact the appointment office or your Consultant's secretary (telephone number at the top of this letter).
- If you would like to know more about Kidney Disease and how you can access support please see <https://www.kidneycareuk.org/> where you will find very useful information, videos and guidance
- Please remember to keep your vaccinations up to date – you should ensure you receive the flu vaccine each year and the pneumococcal vaccine every 5 years. If in doubt check with your GP and if needed your Renal Team.